

ADMISSION NOTE - Patient synth_01 - Acute COPD exacerbation

PATIENT: Smith, John

MRN: 8675309

ADMIT DATE: 2023-10-23

ATTENDING: Dr. L. Martin

CHIEF COMPLAINT: Worsening shortness of breath x 3 days.

HISTORY OF PRESENT ILLNESS: Mr. Smith is a 68-year-old male with a known history of GOLD Stage III COPD who presents to the ED with a three-day history of progressively worsening dyspnea, increased productive cough, and a change in sputum color to yellow-green. He reports that his symptoms are significantly worse than his baseline dyspnea on exertion. He has been using his rescue albuterol inhaler every 2-3 hours with minimal relief. He denies fever, chills, pleuritic chest pain, or hemoptysis. He notes some mild, vague lower abdominal discomfort but denies dysuria, frequency, or urgency.

PAST MEDICAL HISTORY:

1. Chronic Obstructive Pulmonary Disease (COPD), diagnosed 10 years ago
2. Hypertension (HTN)
3. Hyperlipidemia (HLD)

PAST SURGICAL HISTORY: None.

SOCIAL HISTORY: Retired construction worker. 40 pack-year smoking history, quit 5 years ago. Reports 1-2 beers on weekends. Lives at home with his wife, who is supportive.

HOME MEDICATIONS:

- Tiotropium (Spiriva) HandiHaler 18 mcg, one inhalation daily
- Fluticasone/Salmeterol (Advair) 250/50, one puff BID
- Lisinopril 10 mg daily
- Atorvastatin 20 mg daily
- Albuterol MDI PRN

ALLERGIES: No Known Drug Allergies.

REVIEW OF SYSTEMS: As per HPI. Positive for dyspnea, cough, sputum production. Negative for fever, chest pain. Notes mild abdominal discomfort.

PHYSICAL EXAM:

Vitals: T 99.1F, BP 145/88, HR 105, RR 26, SpO2 87% on room air.

General: Alert male in moderate respiratory distress, speaking in 3-4 word sentences.

Lungs: Diffuse expiratory wheezes bilaterally. Diminished breath sounds at bases. Accessory muscle use noted.

Cardiovascular: Tachycardic but regular rhythm. No murmurs, rubs, or gallops.

Abdomen: Soft, non-tender, non-distended. Bowel sounds normoactive.

Extremities: No clubbing, cyanosis, or edema.

ASSESSMENT/PLAN:

1. ****Acute COPD Exacerbation****, likely with infectious trigger given change in sputum. Plan to admit for IV steroids, bronchodilators, and antibiotics. Start O2 via NC to maintain SpO2 >90%.
 - Start Solu-Medrol 125mg IV once, then 60mg IV daily.
 - Albuterol/Ipratropium (Duoneb) nebulizer treatments q4h.
 - Azithromycin 500mg IV daily.
 - Sputum for gram stain and culture.
2. ****Rule out Community Acquired Pneumonia (CAP)****: While likely COPD, will obtain CXR to rule out underlying consolidation.
3. ****Hypertension****: Likely exacerbated by distress. Continue home lisinopril. Monitor BP closely.
4. ****Hyperlipidemia****: Continue home atorvastatin.
5. ****Possible Urinary Tract Infection****: Given mild abdominal discomfort without specific urinary sx. Will monitor. Consider UA if symptoms evolve or if no other source of infection is found.
6. ****Anxiety****: Patient appears anxious related to dyspnea. Start Lorazepam 0.5mg PO BID PRN for anxiety.

PROGRESS NOTE ? Day 1 - Patient synth_01 - Acute COPD exacerbation

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DATE: 2023-10-24

HOSPITAL DAY #2

SUBJECTIVE:

Patient reports feeling 'a bit better' this morning. Less short of breath at rest. Still has a productive cough but feels it's easier to clear secretions. He received one dose of lorazepam overnight and states it helped him rest. Abdominal discomfort has resolved. No chest pain.

OBJECTIVE:

Vitals: T 98.9F, BP 138/82, HR 96, RR 22, SpO2 91% on 2L NC.

Exam: In no acute distress. Lungs with persistent but decreased expiratory wheezing. Improved air movement compared to admission. No accessory muscle use at rest. Heart is tachycardic but regular. Abdomen is soft and non-tender.

Labs/Imaging:

CXR from yesterday showed hyperinflation and flattened diaphragms consistent with COPD, no focal consolidation.

WBC is elevated at 13.5 K/uL. BMP stable.

ASSESSMENT & PLAN:

1. ****Acute COPD Exacerbation****: Showing good response to current therapy. Work of breathing is decreased. Will continue current management.
 - Continue IV Solu-Medrol 60mg daily.
 - Continue Duoneb q4-6h as needed.
 - Continue IV Azithromycin.
 - Titrate O2 to maintain SpO2 90-92%.
2. ****Hypertension****: BP improved. Continue Lisinopril.
3. ****Prophylaxis****: Continue SCDs for VTE prophylaxis.
4. ****Disposition****: Continue to monitor. If he continues to improve, may be able to transition to PO steroids and discharge tomorrow.

PROGRESS NOTE ? Day 2 - Patient synth_01 - Acute COPD exacerbation

PATIENT: Smith, John

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DATE: 2023-10-25

HOSPITAL DAY #3

SUBJECTIVE:

Patient reports significant improvement. 'Feeling much better, ready to go home.' Dyspnea is back to his baseline. Cough is now minimal and sputum is whitish. He is ambulating in the hallways without significant desaturation.

OBJECTIVE:

Vitals: T 98.7F, BP 130/78, HR 88, RR 18, SpO2 93% on room air.

Exam: Alert and oriented, in no distress. Lungs with only faint end-expiratory wheezes. Good air entry throughout. Heart rate and rhythm regular.

Has been weaned off supplemental oxygen successfully.

ASSESSMENT & PLAN:

1. ****Acute COPD Exacerbation****: Resolved. Patient has clinically improved significantly and is stable for discharge.

- Plan for discharge home today.
- Transition to PO prednisone taper (40mg x 5 days, 20mg x 5 days, 10mg x 5 days).
- Complete course of PO Azithromycin (has 2 days left).
- Discontinue IVs and Duonebs.
- Ensure patient has rescue Albuterol inhaler prescription and understands the plan.

2. ****Hypertension****: Well-controlled.

3. ****Discharge Planning****: Patient and wife have received education on medication changes and importance of follow-up. Case management has confirmed safe discharge home. PCP appointment to be scheduled.

Sputum culture sent on admission is still pending.

LABORATORY REPORT - Patient synth_01 - Acute COPD exacerbation

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--- CHEMISTRY (Date: 2023-10-23) ---

Sodium (Na): 138 mmol/L (Ref: 136-145)

Potassium (K): 4.1 mmol/L (Ref: 3.5-5.1)

Chloride (Cl): 101 mmol/L (Ref: 98-107)

CO2: 29 mmol/L (Ref: 22-29)

BUN: 22 mg/dL (Ref: 7-20)

Creatinine (Cr): 1.1 mg/dL (Ref: 0.7-1.3)

Glucose (Gluc): 115 mg/dL (Ref: 70-100)

--- HEMATOLOGY (Date: 2023-10-23) ---

WBC: 13.5 K/uL (Ref: 4.5-11.0)

Hgb: 14.8 g/dL (Ref: 13.5-17.5)

Hct: 44 % (Ref: 41-53)

Platelets: 220 K/uL (Ref: 150-450)

--- MICROBIOLOGY (Date: 2023-10-23) ---

Sputum Gram Stain: Many PMNs, mixed gram-positive cocci in pairs and chains.

Sputum Culture & Sensitivity: Pending.

--- IMAGING (Date: 2023-10-23) ---

CXR Chest PA/Lat: Report reads 'Bilateral hyperinflation and flattened hemidiaphragms consistent with chronic obstructive pulmonary disease. No focal airspace consolidation, pleural effusion, or pneumothorax.'

MEDICATION RECORD - Patient synth_01 - Acute COPD exacerbation

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--- Admission Medications (Home Meds) ---

1. Tiotropium (Spiriva) HandiHaler 18 mcg inhalation daily
2. Fluticasone/Salmeterol (Advair) 250/50 one puff BID
3. Lisinopril 10 mg PO daily
4. Atorvastatin 20 mg PO daily
5. Albuterol MDI 2 puffs q4-6h PRN shortness of breath

--- Inpatient Medication Orders ---

1. Solu-Medrol 60 mg IV daily
2. Azithromycin 500 mg IV daily
3. Albuterol-Ipratropium (Duoneb) 3mL nebulizer q4h
4. Lisinopril 10 mg PO daily
5. Atorvastatin 20 mg PO daily
6. Lorazepam 0.5 mg PO BID PRN anxiety
7. Enoxaparin 40mg SubQ daily for VTE Prophylaxis

--- Discharge Medications ---

1. Azithromycin 500 mg PO daily - take for 2 more days to complete 5-day course.
2. Prednisone 20 mg tablets - Take 40 mg (2 tabs) daily for 5 days, then 20 mg (1 tab) daily for 5 days, then 10 mg (1/2 tab) daily for 5 days, then stop.
3. Resume all home medications:
 - Tiotropium (Spiriva) HandiHaler 18 mcg inhalation daily
 - Fluticasone/Salmeterol (Advair) 250/50 one puff BID
 - Lisinopril 10 mg PO daily
 - Atorvastatin 20 mg PO daily
 - Albuterol MDI 2 puffs q4-6h PRN shortness of breath

DISCHARGE ADVICE - Patient synth_01 - Acute COPD exacerbation

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DISCHARGE DATE: 2023-10-25

DISCHARGE DIAGNOSES:

1. Acute Exacerbation of Chronic Obstructive Pulmonary Disease
2. Hypertension, Chronic
3. Hyperlipidemia
4. Urinary Tract Infection

CONDITION AT DISCHARGE: Stable. Breathing has returned to baseline. Patient is independent with all ADLs.

DISCHARGE MEDICATIONS: You have been given prescriptions for Azithromycin (an antibiotic) and a tapering course of Prednisone (a steroid). Please take them exactly as prescribed. A detailed list is provided on a separate sheet. You should also resume all of your regular home medications.

FOLLOW-UP:

Please schedule a follow-up appointment with your primary care physician, Dr. Davis, in the next 5-7 days to review your progress.

PENDING RESULTS:

The sputum sample we collected on your admission is still being processed in the lab. The culture results are pending. We will call you in 3-4 days with these results, or if any changes to your antibiotic plan are necessary based on the findings.

DIET & ACTIVITY: You may resume your normal diet. Increase your activity slowly as tolerated. Avoid smoke and other lung irritants.